



SUMMARY OF PRODUCT CHARACTERISTICS

AUROZAPINE

Mirtazapine Tablets 15 mg and 30 mg

R, Only

NAME OF DRUG PRODUCT: Mirtazapine Tablets 15 mg
Mirtazapine Tablets 30 mg
(TRADE) NAME OF THE PRODUCT: AUROZAPINE 15
AUROZAPINE 30
STRENGTH: 15 mg and 30 mg
PHARMACEUTICAL DOSAGE FORM: Tablet.

QUALITATIVE AND QUANTITATIVE COMPOSITIONS:
Mirtazapine Tablets 15 mg:
Each film-coated tablet contains Mirtazapine Ph.Eur. 15 mg.
Mirtazapine Tablets 30 mg:
Each film-coated tablet contains Mirtazapine Ph.Eur. 30 mg.

PHARMACEUTICAL FORM:
Mirtazapine Tablets 15 mg: Yellow, biconvex capsule shaped film coated tablets with a scoreline in between 0 and 8 on one side and 'A' debossed on the otherside.
Mirtazapine Tablets 30 mg: Reddish brown, biconvex capsule shaped film coated tablets with a scoreline in between 0 and 9 on one side and 'A' debossed on the otherside.

CLINICAL PARTICULARS:

Therapeutic indications
Episode of major depression.

Posology and method of administration

Adults: Treatment should begin with 15mg daily. The dosage generally needs to be increased to obtain an optimal clinical response. The effective daily dose is usually between 15mg and 45mg (the dose should be taken at night). Mirtazapine begins to exert its effect in general after 1 to 2 weeks of treatment. Treatment with an adequate dose should result in a positive response within 2 to 4 weeks. With an insufficient response, the dose can be increased up to the maximum dose. If there is no response within a further 2 to 4 weeks, then treatment should be stopped.

Elderly: The recommended dose is the same as that for adults. In elderly patients an increase in dosing should be done under close supervision to elicit a satisfactory and safe response.

Children and Adolescents under the age of 18 years: Mirtazapine should not be used in children and adolescents under the age of 18 years.

Renal Impairment: The clearance of Mirtazapine may be decreased in patients with moderate to severe renal impairment (creatine clearance <40 ml/ min). This should be taken into account when prescribing Mirtazapine to this category of patients.

Hepatic Impairment: The clearance of Mirtazapine may be decreased in patients with hepatic impairment. This should be taken into account when prescribing Mirtazapine to this category of patients particularly with severe hepatic impairment, as patients with severe hepatic impairment, as patients with severe hepatic impairment have not been investigated. Mirtazapine has an elimination half-life of 20-40 hours and therefore, Mirtazapine is suitable for once daily administration. It should be taken preferably as a single night-time dose before going to bed. Mirtazapine may also be given in two divided doses (one in the morning and once at night time, the higher dose should be taken at night). The tablets should be taken orally, with fluid, and swallowed without chewing. Patients with depression should be treated for a sufficient period of at least 6 months to ensure that they are free from symptoms. It is recommended to discontinue treatment with Mirtazapine gradually to avoid withdrawal symptoms.

Method of administration:
The tablets should be taken orally, with fluid, and swallowed without chewing.

Contraindications

Hypersensitivity to the active substance or to any of the excipients. Concomitant use of mirtazapine with monoamine oxidase (MAO) inhibitors.

Special warnings and precautions for use

Suicidality in Children and Adolescents

- Antidepressants increase the risk of suicidal thinking and behaviour (suicidality) in children and adolescents with major depressive disorder (MDD) and other psychiatric disorders.-Anyone considering the use of an antidepressant in a child or adolescent for any clinical use must balance the risk of increased suicidality with the clinical need.
- Patients who are started on therapy should be observed closely for clinical worsening, suicidality, or unusual changes in behaviour.
- Families and caregivers should be advised to closely observe the patient and to communicate with the prescriber.
- The indication(s) approved in paediatric for the particular drug should be clearly stated/ included

Suicide/suicidal thoughts or clinical worsening

Depression is associated with an increased risk of suicidal thoughts, self-harm and suicide (suicide related events). This risk persists until significant remission occurs. As improvement may not occur during the first few weeks or more of treatment, patients should be closely monitored until such improvement occurs. It is general clinical experience that the risk of suicide may increase in the early stages of recovery.

Patients with a history of suicide-related events or those exhibiting a significant degree of suicidal ideation prior to commencement of treatment are known to be at greater risk of suicidal thoughts or suicide attempts, and should receive careful monitoring during treatment. A meta-analysis of placebo controlled clinical trials of antidepressants in adult patients with psychiatric disorders showed an increased risk of suicidal behaviour with antidepressants compared to placebo in patients less than 25 years old.

Close supervision of patients and in particular those at high risk should accompany therapy with antidepressants especially in early treatment and following dose changes. Patients (and care givers of patients) should be alerted about the need to monitor for any clinical worsening, suicidal behaviour or thoughts and unusual changes in behaviour and to seek medical advice immediately if these symptoms present. With regard to the chance of suicide, in particular at the beginning of treatment, only a limited number of Mirtazapine film-coated tablets should be given to the patient.

Bone marrow depression

Bone marrow depression, usually presenting as granulocytopenia or agranulocytosis, has been reported during treatment with mirtazapine. Reversible agranulocytosis has been reported as a rare occurrence in clinical studies with mirtazapine. In the post marketing period with mirtazapine very rare cases of agranulocytosis have been reported, mostly reversible, but in some cases fatal. Fatal cases mostly concerned patients with an age above 65. The physician should be alert for symptoms like fever, sore throat, stomatitis or other signs of infection; when such symptoms occur, treatment should be stopped and blood counts taken.

Jaundice

Treatment should be discontinued if jaundice occurs.

Conditions which need supervision

Careful dosing as well as regular and close monitoring is necessary in patients with:

- Epilepsy and organic brain syndrome: Although clinical experience indicates that epileptic seizures are rare during mirtazapine treatment, as with other antidepressants, Mirtazapine should be introduced cautiously in patients who have a history of seizures. Treatment should be discontinued in any patient who develops seizures, or where there is an increase in seizure frequency.
- Hepatic impairment: Following a single 15mg oral dose of mirtazapine, the clearance of mirtazapine was approximately 35% decreased in mild to moderate hepatically impaired patients, compared to subjects with normal hepatic function. 35% decreased in mild to moderate hepatically impaired patients, compared to subjects with normal hepatic function.

The average plasma concentration of mirtazapine was about 55% increased.

- Renal impairment: Following a single 15mg oral dose of mirtazapine, in patients with moderate (creatinine clearance <40ml/min) and severe (creatinine clearance <10ml/min) renal impairment the clearance of mirtazapine was about 30% and 50% decreased respectively, compared to normal subjects. The average plasma concentration of mirtazapine was about 55% and 115% increased respectively. No significant differences were found in patients with mild renal impairment (creatinine clearance <80ml/min) as compared to the control group.

- Cardiac diseases like conduction disturbances, angina pectoris and recent myocardial infarction, where normal precautions should be taken and concomitant medicines carefully administered.
- Low blood pressure
- Diabetes mellitus: In patients with diabetes, antidepressants may alter glycaemic control. Insulin and/or oral hypoglycaemic dosage may need to be adjusted and close monitoring is recommended.

Like with other antidepressants, the following should be taken into account:

- Worsening of psychotic symptoms can occur when antidepressants are administered to patients with schizophrenia or other psychotic disturbances; paranoid thoughts can be intensified
- When the depressive phase of bipolar disorder is being treated, it can transform into the manic phase. Patients with a history of mania/hypomania should be closely monitored. Mirtazapine should be discontinued in any patient entering a manic phase.
- Although mirtazapine is not addictive, post-marketing experience shows that abrupt termination of treatment after long term administration may sometimes result in withdrawal symptoms. The majority of withdrawal reactions are mild and self-limiting. Among the various reported withdrawal symptoms, dizziness, agitation, anxiety, headache and nausea are the most frequently reported. Even though they have been reported as withdrawal symptoms, it should be realized that these symptoms may be related to the underlying disease. As advised in section 4.2, it is recommended to discontinue treatment with mirtazapine gradually.
- Care should be taken in patients with micturition disturbances like prostate hypertrophy and in patients with acute narrow-angle glaucoma and increased intra-ocular pressure (although there is little chance of problems with Mirtazapine because of its very weak anticholinergic activity).
- Akathisia/psychomotor restlessness: The use of antidepressants have been associated with the development of akathisia, characterised by a subjectively unpleasant or distressing restlessness and need to move often accompanied by an inability to sit or stand still. This is most likely to occur within the first few weeks of treatment. In patients who develop these symptoms, increasing the dose may be detrimental.

Hyponatraemia

Hyponatraemia, probably due to inappropriate antidiuretic hormone secretion (SIADH), has been reported very rarely with the use of mirtazapine. Caution should be exercised in patients at risk, such as elderly patients or patients concomitantly treated with medications known to cause hyponatraemia

Serotonin syndrome

Interaction with serotonergic active substances: serotonin syndrome may occur when selective serotonin reuptake inhibitors (SSRIs) are used concomitantly with other serotonergic active substances. Symptoms of serotonin syndrome may be hyperthermia, rigidity, myoclonus, autonomic instability with possible rapid fluctuations of vital signs, mental status changes that include confusion, irritability and extreme agitation progressing to delirium and coma. Caution should be advised and a closer clinical monitoring is required when these active substances are combined with mirtazapine. Treatment with mirtazapine should be discontinued if such events occur and supportive symptomatic treatment initiated. From post marketing experience it appears that serotonin syndrome occurs very rarely in patients treated with mirtazapine alone

Elderly patients

Elderly patients are often more sensitive, especially with regard to the undesirable effects of antidepressants. During clinical research with mirtazapine, undesirable effects have not been reported more often in elderly patients than in other age groups.

Lactose

This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose galactose malabsorption should not take this medicine.

Effects on the Ability to Drive or Operate Machinery: Aurozapine may impair concentration and alertness. Patients treated with antidepressants should avoid the performance of potentially dangerous tasks, which require alertness and good concentration e.g., driving a motor vehicle or operating machinery.

Pregnancy and Lactation

Pregnancy

Limited data of the use of mirtazapine in pregnant women do not indicate an increased risk for congenital malformations.

Studies in animals have not shown any teratogenic effects of clinical relevance, however developmental toxicity has been observed. Caution should be exercised when prescribing to pregnant women. If Mirtazapine is used until, or shortly before birth, postnatal monitoring of the newborn is recommended to account for possible discontinuation effects.

Epidemiological data have suggested that the use of SSRIs in pregnancy, particularly in late pregnancy, may increase the risk of persistent pulmonary hypertension in the newborn (PPHN). Although no studies have investigated the association of PPHN to mirtazapine treatment, this potential risk cannot be ruled out taking into account the related mechanism of action (increase in serotonin concentrations).

P1533880

A/s: 165 x 430 mm ■ Black

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		Aurozapine Tablets	Leaflet	P1533880	07.04.2023 & 10.45 am
		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
Malaysia U3	01	NEW			
Team Leader	Kiran	Dimensions		No. of Colours : 01	
Initiator	Shirisha	165 x 430 mm			
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